



250000508610777

Name : Ms. SOUMAYA DANIEL
Age / Gender : 47 Year(s)/ Female
Contact No. : +919810577702
Address : HOUSE NO 604, MILLENNIUM TOWER, AT -...
Pin code : 121002

VID No. : 250000508610777
PID No. : P41525553555607
Referred by : SELF
Sample Collected At : Hcl Avitas Private Limited, 806,siddharth 96 Nehru Place,110019

SUMMARY REPORT

Investigation Outside Reference Range (Abnormal)

Investigation	Observed Value	Unit	Biological Reference Interval
ESR (Erythrocyte Sedimentation Rate) (EDTA Whole Blood)	34	mm/hr	<= 20
HbA1C- Glycated Haemoglobin	5.8	%	Non-diabetic: <= 5.6 Pre-diabetic: 5.7-6.4 Diabetic: >= 6.5
Non HDL Cholesterol	145.5	mg/dL	Optimal <130 ,Desirable 130-159 ,Borderline high 159-189 ,High 189-220 ,Very High >=220
LDL Cholesterol	125.74	mg/dL	Optimal <100 ,Near Optimal 100-129 ,Borderline high 130-159 ,High 160-189 ,Very High >=190
Gamma GT (GGTP) (Gamma-Glutamyl Carboxy Nitroanilide (GGCNA) - IFCC method)	66	U/L	9-36
LDH (Lactate Dehydrogenase), Serum (Serum Lactate-pyruvate IFCC)	258	U/L	125-220
Vitamin D Total - 25 Hydroxy (OH) (Serum Electrochemiluminescence immunoassay (ECLIA))	28.1	ng/mL	Deficiency: < 10 Insufficiency: 10-30 Sufficiency: 30-100 Hypervitaminosis: > 100
Vitamin B12 (Cyanocobalamin) (Serum Electrochemiluminescence immunoassay (ECLIA))	191	pg/mL	197-771

Investigation Within Reference Range (Normal)

MEDICAL LABORATORY REPORT

Registered On: 11/09/2025 1:03 PM

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Reported On: 11/09/2025 4:45 PM

PROCESSING LOCATION :

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CBC Haemogram
Thyroid Profile - 1
Iron Studies
Glucose Fasting
RA (Rheumatoid Arthritis) Factor, Serum
Renal Function Test(RFT)
GFR with Creatinine (Above 18 Years)

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CBC Haemogram

Investigation	Observed Value	Unit	Biological Reference Interval
<u>Erythrocytes</u>			
Haemoglobin (Hb) (Spectrophotometry)	13.7	gm/dL	12.5-16
Erythrocyte (RBC) Count	4.71	mill/cu.mm	4.2-5.4
HCT(Hematocrit)	41.9	%	37-47
MCV (Mean Corpuscular Volume)	88.9	fL	78-100
MCH (Mean Corpuscular Hb)	29.2	pg	27-31
MCHC (Mean Corpuscular Hb Concn.)	32.8	gm/dL	32-36
RDW (Red Cell Distribution Width)	13.7	CV%	11.5-14.0
<u>Leucocytes</u>			
Total Leucocytes (WBC) Count	6100	cells/cu.mm	4300-10300
Absolute Neutrophils Count	3373	cells/cu.mm	2000-7000
Absolute Lymphocyte Count	2178	cells/cu.mm	1000-3000
Absolute Monocyte Count	256	cells/cu.mm	200-1000
Absolute Eosinophils Count	207	cells/cu.mm	20-500
Absolute Basophils Count	85	cells/cu.mm	20-100
Neutrophils	55.3	%	40-80
Lymphocytes	35.7	%	20-40
Monocytes	4.2	%	2.0-10
Eosinophils	3.4	%	1-6
Basophils	1.4	%	0-2
<u>Platelets</u>			
Platelet count	436	10 ³ /μL	150-450
MPV (Mean Platelet Volume)	7.0	fL	6-9.5
PCT (Platelet Haematocrit)	0.30	%	0.2-0.5
PDW (Platelet Distribution Width)	15.9	%	9-17

EDTA Whole Blood

: Test is done on Automated Five Part Cell Counter. Hemoglobin is measured by Spectrophotometry method. WBC, RBC and Platelet Count are measured by Coulter Principle (Impedance Method). WBC Differential is done by VCS Method. MCV and RDW are derived from RBC histogram. MPV and PDW are derived from Platelet histogram. Calculated Parameters are: HCT, MCH, MCHC, PCT and Absolute WBC counts. All abnormal hemogram are reviewed and confirmed microscopically. Differential count is based on approximately 10,000 cells.

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Reg No.DMC - 5204

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Investigation	Observed Value	Unit	Biological Reference Interval
 LDH (Lactate Dehydrogenase), Serum (Serum,Lactate-pyruvate IFCC)	258	U/L	125-220
 RA (Rheumatoid Arthritis) Factor, Serum (Serum,Immunoturbidimetry)	10.4	IU/mL	Non reactive: < 14

Interpretation :

- The detection of rheumatoid factor (RF) is one of the criteria of the American Rheumatism Association (ARA) for the diagnosis of Rheumatoid Arthritis (RA).
- RFs play an important role in the differential diagnosis between RA and other rheumatic diseases. They also permit prognostic statements with regards to RA.

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Investigation

Glucose Fasting

(Plasma-F,Hexokinase)

Observed Value

83.2

Unit

mg/dL

Biological Reference Interval

Normal: 70-99
 Impaired Tolerance: 100-125
 Diabetes mellitus: ≥ 126
 (on more than one occasion)
 (American diabetes association
 guidelines 2021)

Lipid Profile - 2 (Mini - Fasting)

Cholesterol Total, Serum

(Serum,Cholesterol oxidase-peroxidase (CHOD-
 POD))

194.1

mg/dL

Desirable: < 200
 Borderline High: 200-239
 High: ≥ 240

Triglycerides, Serum

(Serum,Glycerol-3-phosphate peroxidase
 chromogenic method (GPO-POD))

98.8

mg/dL

Normal: < 150
 Borderline High: 150-199
 High: 200-499
 Very High: ≥ 500

HDL Cholesterol Direct

(Serum,Accelerator Selective Detergent)

48.6

mg/dL

> 40

Non HDL Cholesterol

(Serum,Calculated)

145.5

mg/dL

Optimal <130
 ,Desirable 130-159
 ,Borderline high 159-189
 ,High 189-220
 ,Very High ≥ 220

LDL Cholesterol

(Serum,Calculated)

125.74

mg/dL

Optimal <100
 ,Near Optimal 100-129
 ,Borderline high 130-159
 ,High 160-189
 ,Very High ≥ 190

VLDL Cholesterol

(Serum,Calculated)

20

mg/dL

<30

LDL/HDL Ratio

(Serum,Calculated)

3

2.5-3.5

Cholestrol / HDL Ratio

(Serum,Calculated)

4

3.5-5.0

Note: Reference Interval as per National Cholesterol Education Program (NCEP) Adult Treatment Panel III Report.

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Investigation	Observed Value	Unit	Biological Reference Interval
<u>Liver Function Test(LFT)</u>			
 SGPT (ALT) (Serum,IFCC w/o pyridoxal phosphate activation)	27.6	U/L	0-34
 SGOT (AST) (Serum,IFCC w/o pyridoxal phosphate activation)	22.0	U/L	0-31
 Alkaline Phosphatase, Serum (Serum,p-nitrophenyl phosphate (pNPP))	103	U/L	35-104
 <u>Bilirubin Total, Direct, Indirect</u>			
Bilirubin Total (Serum,Diazo method)	0.51	mg/dL	0.2-1.2
Bilirubin Direct (Serum,Diazo method)	0.18	mg/dL	0.0-0.5
Bilirubin- Indirect (Serum,Calculated)	0.33	mg/dL	0.1-1.0
 Gamma GT (GGTP) (Serum,Gamma-Glutamyl Carboxy Nitroanilide (GGCNA) - IFCC method)	66	U/L	9-36
 <u>Proteins, Serum</u>			
Total Protein (Serum,Biuret test)	7.45	gm/dL	6.4-8.3
Albumin, Serum (Serum,Bromocresol Green (BCG))	4.37	gm/dL	3.5-5.2
Globulin (Serum,Calculated)	3.08	gm/dL	1.8-3.6
Albumin/Globulin Ratio (Serum,Calculated)	1.42		1.1-2.2
 <u>Renal Function Test(RFT)</u>			
Uric Acid, Serum (Serum,Uricase)	5.4	mg/dL	2.6-6
BUN, Serum (Serum,Urease)	9.8	mg/dL	6-20

Remark: In blood, Urea is usually reported as BUN and expressed in mg/dl. BUN mass units can be converted to urea mass units by multiplying by 2.14.



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Investigation	Observed Value	Unit	Biological Reference Interval
Calcium, Serum (Serum,Arsenazo III)	9.56	mg/dL	8.4-10.2
Creatinine, Serum (Serum,Modified Jaffes method)	0.78	mg/dL	0.50-0.90
Phosphorus, Serum (Serum,Molybdate UV)	4.68	mg/dL	2.3-4.7
 <u>Electrolytes, Serum</u> (Serum , ISE Indirect)			
Sodium, Serum	138	mEq/L	136-145
Potassium, Serum	4.00	mEq/L	3.5-5.1
Chloride, Serum	98.3	mEq/L	98-107
 <u>GFR with Creatinine (Above 18 Years)</u>			
Creatinine, Serum (Serum,Modified Jaffes method)	0.78	mg/dL	0.50-0.90
eGFR-Estimated Glomerular Filtration Rate (Serum)	94	mL/min/1.73m ²	Normal or high ≥ 90 ,Mild decrease 60-89 ,Moderate decrease 45-59 ,Moderate to severe decrease 30-44 ,Severe decrease 15-29 ,Kidney failure <15

Remarks :

- eGFR : Calculated by MDRD formula (Race considered = Non black) .
- Kidney damage is defined as abnormalities on pathological, urine, blood or imaging tests. Adapted from Levey. et. al
- Equation is not valid for patients below 18 years of age.Calculated by IDMS-Traceable CKD-EPI creatinine equation.

Reference:

Classification of Chronic renal Disease ; National Kidney Foundation Kidney Disease Outcome Quality Initiative (NKF KDOQI) Clinical Practice guidelines .

 Iron Studies

Iron, Serum (Serum,FerroZine)	103.3	µg/dL	33-193
UIBC (Unsaturated Iron Binding Capacity) (Serum,FerroZine)	260.9	µg/dL	135-392

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Investigation	Observed Value	Unit	Biological Reference Interval
TIBC (Serum,Calculated)	364	µg/dL	250-450
Transferrin Saturation (Serum,Calculated)	28	%	14-50

Interpretation :

- Measurements of serum iron, TIBC and the percentage of iron saturation of transferrin are useful screening tests for iron deficiency anaemia.
- However, serum iron exhibits significant diurnal variation and may transiently rise or reach reference values after dietary or iron supplements & post blood transfusion.
- The diagnostic specificity of a low serum iron for iron deficiency is lost in the presence of acute & chronic inflammatory processes as the concentrations of iron and transferrin in the serum are significantly affected, and fall rapidly as part of the acute phase response irrespective of the iron stores status in the body.
- Hence, Concurrent measurement of the markers mentioned in the below interpretative table alongwith serum iron studies improves the diagnostic specificity for iron deficiency anaemia & also provides a reliable work up for microcytic hypochromic anaemia.

Tests	Iron Deficiency anaemia	Anaemia of Chronic disease	Iron overload	Hemoglobinopathy (Especially Trait)
Serum Iron	Decreased	Decreased	Increased	Normal
Serum Total Iron Binding Capacity	Increased	Decreased or Normal	Increased or Normal	Normal
% Transferrin Saturation	Decreased	Decreased or Normal	Increased or Normal	Normal
Serum Ferritin	Decreased	Increased	Increased or Normal	Normal
Serum Soluble Transferrin receptor	Increased	Normal	Decreased	Normal
Serum Hepcidin	Normal	Increased	Normal	Normal

Associated Tests :

- Serum Soluble Transferrin receptor
- Serum Hepcidin

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Investigation**Observed Value****Unit****Biological Reference Interval****HCL_25NG-home visit****ESR (Erythrocyte Sedimentation Rate) 34**

mm/hr

<= 20

(EDTA Whole Blood)

Method: Photometry Capillary**Interpretation:**

- It indicates presence and intensity of an inflammatory process, never diagnostic of a specific disease. Changes are more significant than a single abnormal test.
- It is a prognostic test and used to monitor the course or response to treatment of diseases like tuberculosis, bacterial endocarditis, acute rheumatic fever, rheumatoid arthritis, SLE, Hodgkins disease, temporal arteritis, polymyalgia rheumatica.
- It is also increased in pregnancy, multiple myeloma, menstruation, and hypothyroidism.

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Investigation**Vitamin D Total - 25 Hydroxy (OH)**

(Serum,Electrochemiluminescence immunoassay
 (ECLIA))

Observed Value**28.1****Unit**

ng/mL

Biological Reference Interval

Deficiency: < 10
 Insufficiency: 10-30
 Sufficiency: 30-100
 Hypervitaminosis: > 100

Interpretation :

- Vitamin D is a fat soluble vitamin and exists in two main forms as cholecalciferol(vitamin D3) which is synthesized in skin from 7-dehydrocholesterol in response to sunlight exposure & Ergocalciferol(vitamin D2) present mainly in dietary sources.Both cholecalciferol & Ergocalciferol are converted to 25(OH)vitamin D in liver.
- Testing for 25(OH)vitamin D is recommended as it is the best indicator of vitamin D nutritional status as obtained from sunlight exposure & dietary intake. For diagnosis of vitamin D deficiency it is recommended to have clinical correlation with serum 25(OH)vitamin D, serum calcium, serum PTH & serum alkaline phosphatase.
- During monitoring of oral vitamin D therapy- suggested testing of serum 25(OH)vitamin D is after 12 weeks or 3 mths of treatment. However, the required dosage of vitamin D supplements & time to achieve sufficient vitamin D levels show significant seasonal(especially winter) & individual variability depending on age, body fat, sun exposure, physical activity ,genetic factors(especially variable vitamin D receptor responses), associated liver or renal disease, malabsorption syndromes and calcium or magnesium deficiency influencing the vitamin D metabolism Vitamin D toxicity is known but very rare.kindly correlate clinically, repeat with fresh sample if indicated.

Abbreviation :

ECLIA : Electrochemiluminescence Immunoassay

Associated Test Profile :

- For diagnosis of vitamin D deficiency it is recommended to have clinical correlation with serum 25(OH)vitamin D and serum PTH.An inverse relationship exists between PTH and 25(OH)D levels, Parathyroid hormone levels start to rise at 25(OH)D levels below 31 ng/mL & usually decrease after the correction of vitamin D insufficiency.Thus, restoration of PTH and 25(OH)D levels to normalcy after adequate vitamin D replacement therapy is a useful monitoring strategy.
- As a holistic & scientific approach for diagnosis and optimal treatment for vitamin D deficiency, Vitamin D plus profile (25 Hydroxy(OH) Vit D and PTH) is suggested.

Vitamin B12 (Cyanocobalamin)**191**

pg/mL

197-771

(Serum,Electrochemiluminescence immunoassay
 (ECLIA))

Interpretation :

- Vit B12 levels are decreased in megaloblastic anemia, partial/total gastrectomy, pernicious anemia, peripheral neuropathies, chronic alcoholism, senile dementia, and treated epilepsy.
- An associated increase in homocysteine levels is an independent risk marker for cardiovascular disease and deep vein thrombosis.

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Investigation

Observed Value

Unit

Biological Reference Interval

HoloTranscobalamin II levels are a more accurate marker of active VitB12 component.

Abbreviation :

ECLIA : Electrochemiluminescence Immunoassay

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Observed Value

Unit

Biological Reference Interval

**HCL 25NG-home visit****Thyroid Profile - 1**

(Serum, Electrochemiluminescence immunoassay (ECLIA))

T3 (Total Triiodothyronine)	146	ng/dL	84.6-201.8 Second Trimester : 128.9 - 262.3 First Trimester : 104.8 - 229.8 Third trimesters : 135.4 - 261.7
T4 (Total Thyroxine)	10.0	µg/dL	5.1-14.1 First Trimester : 7.33 - 14.8 Second Trimester : 7.93 - 16.1 Third Trimester : 6.95 - 15.7
TSH (Thyroid Stimulating Hormone) - Ultrasensitive, Serum	1.900	µIU/mL	0.54-5.3 First Trimester : 0.33-4.59 Second Trimester : 0.35-4.10 Third trimester : 0.21-3.15

INTERPRETATION

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TSH	T3 / FT3	T4 / FT4	Suggested Interpretation for the Thyroid Function Tests Pattern
Within Range	Decreased	Within Range	Isolated Low T3 - often seen in elderly & associated Non-Thyroidal illness. In elderly, the drop in T3 level can be up to 25%.
Raised	Within Range	Within Range	- Isolated High TSH, especially in the range of 4.7 to 15 mIU/ml, is commonly associated with Physiological & Biological TSH Variability. - Subclinical Autoimmune Hypothyroidism - Intermittent T4 therapy for hypothyroidism - Recovery phase after Non-Thyroidal illness
Raised	Decreased	Decreased	- Chronic Autoimmune Thyroiditis - Post thyroidectomy, Post radioiodine - Hypothyroid phase of transient thyroiditis
Raised or within Range	Raised	Raised or within Range	- Interfering antibodies to thyroid hormones (anti-TPO antibodies) - Intermittent T4 therapy or T4 overdose - Drug interference - Amiodarone, Heparin, Beta blockers, steroids, anti-epileptics
Decreased	Raised or within Range	Raised or within Range	- Isolated Low TSH - especially in the range of 0.1 to 0.4 often seen in elderly & associated with Non-Thyroidal illness - Subclinical Hyperthyroidism - Thyroxine ingestion
Decreased	Decreased	Decreased	- Central Hypothyroidism - Non-Thyroidal illness - Recent treatment for Hyperthyroidism (TSH remains suppressed)
Decreased	Raised	Raised	- Primary Hyperthyroidism (Graves' disease), Multinodular goitre, Toxic nodule - Transient thyroiditis: Postpartum, Silent (lymphocytic), Postviral (granulomatous, subacute, DeQuervain's), Gestational thyrotoxicosis with hyperemesis gravidarum
Decreased or within Range	Raised	Within Range	- T3 toxicosis - Non-Thyroidal illness

References: 1. Interpretation of thyroid function tests. Dayan et al. THE LANCET • Vol 357 • February 24, 2001
 2. Laboratory Evaluation of Thyroid Function, Indian Thyroid Guidelines, JAPI, January 2011, vol. 59

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SAMPLE PROCESSED AT: METROPOLIS HEALTHCARE LTD E-21,
 B1 MOHAN CO-OP IND ESTATE NEW DELHI-110044



250000508610777

Name : Ms. SOUMAYA DANIEL
 Age / Gender : 47 Year(s)/ Female
 Contact No. : +919810577702
 Address : HOUSE NO 604, MILLENNIUM TOWER, AT -...
 Pin code : 121002

VID No. : 250000508610777
 PID No. : P41525553555607
 Referred by : SELF
 Sample Collected At : Hcl Avitas Private Limited, 806,siddharth 96 Nehru Place,110019

Investigation**Observed Value****Unit****Biological Reference Interval****HCL 25NG-home visit****HbA1c- Glycated Haemoglobin**

(EDTA Whole Blood)

HbA1C- Glycated Haemoglobin

(High-Performance Liquid Chromatography (HPLC))

5.8**%**

Non-diabetic: <= 5.6

Pre-diabetic: 5.7-6.4

Diabetic: >= 6.5

Estimated Average Glucose (eAG)

(Calculated)

120**mg/dL****Interpretation & Remark:**

1. HbA1c is used for monitoring diabetic control. It reflects the estimated average glucose (eAG).
2. HbA1c has been endorsed by clinical groups & ADA (American Diabetes Association) guidelines 2022, for diagnosis of diabetes using a cut-off point of 6.5%.
3. Trends in HbA1c are a better indicator of diabetic control than a solitary test.
4. Low glycated haemoglobin(below 4%) in a non-diabetic individual are often associated with systemic inflammatory diseases, chronic anaemia(especially severe iron deficiency & haemolytic), chronic renal failure and liver diseases. Clinical correlation suggested.
5. To estimate the eAG from the HbA1C value, the following equation is used: $eAG(mg/dl) = 28.7 \times A1c - 46.7$
6. Interference of Haemoglobinopathies in HbA1c estimation.

A. For HbF > 25%, an alternate platform (Fructosamine) is recommended for testing of HbA1c.

B. Homozygous hemoglobinopathy is detected, fructosamine is recommended for monitoring diabetic status

C. Heterozygous state detected (D10/ turbo is corrected for HbS and HbC trait).

7. In known diabetic patients, following values can be considered as a tool for monitoring the glycemic control. Excellent Control - 6 to 7 %, Fair to Good Control - 7 to 8 %, Unsatisfactory Control - 8 to 10 % and Poor Control - More than 10 % .

Note : Hemoglobin electrophoresis (HPLC method) is recommended for detecting hemoglobinopathy.

Dr. GEETA CHOPRA
 MD (Pathology)
 Regional Chief Of Lab
 Reg No.DMC - 5204

MEDICAL LABORATORY REPORT

Registered On: 11/09/2025 1:03 PM

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**Routine Examination Profile - Urine**

Investigation	Observed Value	Unit	Biological Reference Interval
<u>HCL 25NG-home visit</u>			
<u>Routine Examination Profile - Urine</u>			
<u>General Examination</u>			
Colour	Pale Yellow		Pale Yellow
Transparency (Appearance)	Slightly Hazy		Clear
Reaction (pH)	6.0		4.5-7.0
Specific gravity	1.015		1.005-1.030
<u>Chemical Examination</u>			
Urine Protein (Albumin)	Absent		Absent
Urine Ketones (Acetone)	Absent		Absent
Urine Glucose (sugar)	Absent		Absent
Bile pigments	Absent		Absent
Bile salts	Absent		Absent
Urobilinogen	Normal		Normal
Nitrite	Negative		Negative
<u>Microscopic Examination</u>			
Red blood cells	1-2	/hpf	0-4
Pus cells (WBCs)	6-8	/hpf	0-9
Epithelial cells	8-10	/hpf	0-4
Crystals	Absent		Absent
Cast	Absent		Absent
Bacteria	Absent		Absent
Trichomonas Vaginalis	Absent		Absent
Yeast cells	Absent		Absent

Note :1

.Chemical examination through Dipstick includes test methods as Protein (Protein Error Principle), Glucose (Glucose oxidase-Peroxidase), Ketone (Legals Test), Bilirubin (Azo- Diazo reaction),Urobilinogen (Diazonium ion Reaction) Nitrite (Griess Method). Abnormal results of chemical examination are confirmed by manual methods.

Dr. FIRDAUS.ALI
M.D (Pathology)
Consultant Pathologist
Reg No.DMC-61238

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Investigation**Observed Value****Unit****Biological Reference Interval****2.**

Negative nitrite test does not exclude the urinary tract infections, Trace proteinuria can be seen with many physiological conditions like prolonged recumbency, exercise, high protein diet. False positive reactions for bile pigments, proteins, glucose and nitrites can be caused by peroxidase like activity by disinfectants, therapeutic dyes, ascorbic acid and certain drugs.

3.

Pre-test conditions : Void first urine ,collect mid-stream urine in clean sterile container to avoid contamination with perineal, vaginal or urethral discharge.

-- End of Report --

MC-2676

Test Marked with NABL symbol are in the
scope of accreditation

Dr. FIRDAUS ALI
M.D (Pathology)
Consultant Pathologist
Reg No.DMC-61238

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